

CITYCARE GENERAL HOSPITAL

OUTPATIENT DEPARTMENT STANDARD
OPERATING PROCEDURE

OPD Registration, Consultation & Follow-up Management

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Department / Unit	Outpatient Department (OPD) / All Specialty Clinics
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1. PURPOSE & SCOPE

This SOP governs the operation of the CityHospital Outpatient Department across all specialty clinics. The OPD serves as the primary interface between the hospital and the community, providing consultations, follow-ups, minor procedures, and diagnostic services for non-hospitalised patients.

- Provide accessible, efficient, and patient-centred outpatient services across all specialties.
- Minimise patient waiting times while maintaining the highest clinical quality standards.
- Ensure complete and accurate documentation for every OPD encounter in the HIS.
- Facilitate seamless transitions between OPD, diagnostics, pharmacy, and inpatient services.
- Support telemedicine services for eligible patients to improve access and convenience.
- Comply with all applicable national and accreditation OPD quality standards.

2. DEFINITIONS & ABBREVIATIONS

Term / Abbreviation	Definition
OPD	Outpatient Department — hospital facility providing care without inpatient admission.
New Patient	First visit to CityHospital or first visit to a specific clinical specialty.
Follow-up Patient	Returning patient with existing MRN and established treatment relationship.
Walk-in Patient	Patient attending OPD without a prior appointment.
Referral	Directed consultation from another physician or healthcare facility.
OPD Number	Unique encounter identifier generated for each individual OPD visit.
DNA	Did Not Attend — patient failed to attend a booked appointment without prior notice.
HIS	Hospital Information System — the electronic platform used for clinical and administrative records.
SOAP Note	Structured clinical note format: Subjective, Objective, Assessment, Plan.
AIDET	Communication framework: Acknowledge, Introduce, Duration, Explanation, Thank You.
NABH	National Accreditation Board for Hospitals and Healthcare Providers.

3. ROLES & RESPONSIBILITIES

Role	Department	Responsibilities
OPD Clerk / Receptionist	OPD Reception	Registration, appointment management, billing initiation, queue token issue
OPD Triage Nurse	OPD Nursing	Vital signs, clinical urgency assessment, patient queue prioritisation
Specialist Physician	Relevant Clinic	Consultation, diagnosis, prescriptions, referrals, SOAP documentation
OPD Nurse	OPD Nursing	Assist consultations, perform minor procedures, patient education
OPD Pharmacist	OPD Pharmacy	Prescription dispensing, patient counselling, medication stock management
OPD Manager	OPD Administration	Patient flow, staff allocation, complaint resolution, KPI monitoring
Radiographer / Lab Tech	Diagnostics	Process and report investigations requested during OPD visit

4. APPOINTMENT SCHEDULING & PATIENT REGISTRATION

4.1 Appointment Scheduling

- Appointments available via hospital website, mobile app, telephone helpline (06:00–22:00), and OPD walk-in desk.
- Appointment slots allocated by specialty, named physician, and session type (new patient or follow-up).
- Appointment confirmation sent by SMS and email at least 24 hours before the visit, including clinic location and preparation requirements.
- Cancellation and rescheduling: patients may cancel up to 2 hours before the appointment via app or helpline.
- DNA rate tracked monthly by specialty; patients with two or more DNAs contacted by OPD coordinator.
- Walk-in patients accommodated in available slots; if none available, next available appointment offered or token issued.

4.2 Patient Registration Workflow

Step 1. Patient presents at OPD reception counter; photo ID and insurance or payment card presented.

Step 2. HIS searched for existing MRN; new MRN created with full demographic data for first-time patients.

Step 3. Chief complaint recorded; specialty and preferred physician confirmed or assigned by OPD coordinator.

Step 4. OPD encounter number generated; queue token issued with estimated wait time displayed on screen.

Step 5. Billing type confirmed: insurance pre-authorisation obtained before consultation if required by insurer.

Step 6. Patient directed to appropriate clinic waiting area with clear wayfinding instructions.

NOTE: All patient data must be verified at each visit. Demographic updates (address, contact number, insurance) must be made in HIS at the point of registration.

5. CONSULTATION WORKFLOW

- Step 1.** OPD nurse calls patient from waiting area by name and token number; guides to pre-consultation assessment bay.
- Step 2.** Nurse records and documents vital signs: BP, HR, RR, SpO2, weight, temperature, and pain score (0–10 NRS).
- Step 3.** Chief complaint confirmed; prior medical records and investigation results loaded in HIS for physician review.
- Step 4.** Physician conducts full consultation: history taking, clinical examination, assessment, and management plan documented as SOAP note.
- Step 5.** Physician enters investigation orders, prescriptions, or procedure requests directly into HIS.
- Step 6.** Consultation note completed in HIS before patient leaves the clinic bay — no retrospective entries.
- Step 7.** Patient counselled on diagnosis, treatment plan, medications, and lifestyle modifications in plain language.
- Step 8.** Follow-up appointment booked and confirmed before patient leaves; printed appointment slip provided.
- Step 9.** Referral letters generated for inter-specialty or community referrals; sent electronically with patient consent.

5.1 Consultation Time Standards

Consultation Type	Allocated Slot Time	Waiting Time Target
New patient consultation	15–20 minutes	30 minutes from registration to physician
Follow-up consultation	10–15 minutes	30 minutes from registration to physician
Procedure or minor surgery	30–60 minutes	As per pre-booked slot
Walk-in urgent assessment	Priority slot	Within 45 minutes of triage

NOTE: If physician is delayed beyond 30 minutes, the OPD coordinator must notify waiting patients, document the delay in HIS, and offer rescheduling if requested.

6. DIAGNOSTIC & INVESTIGATION MANAGEMENT

6.1 Same-Day OPD Investigations

- Physician marks investigations as 'OPD STAT' in HIS for same-day processing and prioritisation.
- Patient directed to phlebotomy, radiology, ECG, or other relevant department with the electronic request.
- Routine OPD blood results turnaround target: within 2 hours of sample collection.
- STAT results turnaround target: within 45 minutes of sample collection.
- Radiograph or ultrasound: dedicated OPD imaging slots reserved each session; plain film reports within 1 hour.
- Results reviewed by physician on patient's return or communicated via telephone for urgent or unexpected findings.

6.2 Critical or Unexpected Results

- Critical results telephoned immediately by lab or radiology to the referring OPD physician.
- Physician decides on action: same-day review in clinic, urgent ED referral, or planned admission.
- If patient has left the building, OPD nurse contacts patient by phone; outcome and action documented in HIS.
- All critical result communications documented with date, time, person notified, and action taken.

6.3 Pre-Booked Investigations

- Investigations ordered at a previous visit must be available for review before the follow-up appointment.
- Patient instructed at time of booking on any preparation requirements: fasting, stopping medications, contrast consent.
- OPD coordinator tracks outstanding pre-booked results and alerts physician if unavailable 24 hours before the appointment.

7. PRESCRIPTION & PHARMACY LIAISON

- Step 1.** Physician issues all prescriptions electronically via HIS; patient receives a printed collection token.
- Step 2.** Patient presents token at OPD pharmacy counter; no handwritten prescriptions accepted except during HIS downtime.
- Step 3.** Pharmacist verifies prescription: legibility, appropriate dose, drug interactions, and allergy cross-check against EMR.
- Step 4.** Medications dispensed; pharmacist counsels patient on correct dosing, storage requirements, and common side effects.
- Step 5.** High-risk medications (warfarin, insulin, opioids, lithium, immunosuppressants): enhanced one-to-one counselling session; monitoring plan discussed and documented.
- Step 6.** Patient signs the dispensing record confirming receipt and counselling; copy retained in pharmacy system.
- Step 7.** Out-of-stock medications: pharmacist provides a signed prescription copy; patient assisted with nearest alternative supplier.

NOTE: Pharmacist must verify every prescription against the patient's full medication list in HIS to identify interactions, duplications, or allergy risks before dispensing.

7.1 Medication Reconciliation in OPD

- At every OPD visit, the physician or nurse reviews and updates the patient's active medication list in HIS.
- Any changes to chronic medications communicated to the patient's GP by letter or HIS electronic notification.
- Patients on high-risk long-term medications (anticoagulants, DMARDs, immunosuppressants) must have current monitoring results on file before prescription is issued.

8. PATIENT FLOW & QUEUE MANAGEMENT

8.1 Token-Based Queue System

- Digital display boards in all waiting areas show current token number being called and estimated wait time.
- Priority queue lanes designated for elderly patients over 70, pregnant women, wheelchair users, and children under 5.
- OPD triage nurse monitors the waiting area continuously; any clinically deteriorating patient escalated immediately to the emergency pathway.
- Physicians required to maintain a maximum 3-token backlog; delays beyond this trigger OPD manager notification.
- Patient satisfaction with waiting time collected via digital feedback kiosk; reviewed weekly by OPD manager.

8.2 Surge & Overflow Management

- Walk-in patient surge: OPD manager activates overflow protocol; additional physician and nurse session opened.
- If aggregate waiting time exceeds 60 minutes: patients proactively informed; intake reviewed; OPD manager approval required to continue accepting walk-ins.
- Seasonal and epidemic demand planning: OPD staffing and slot capacity modelled and adjusted 2 weeks in advance.
- Flu season and holiday contingency plans reviewed and approved by medical director annually.

8.3 No-Show & Cancellation Management

- DNA slots filled from the same-day waiting list or walk-in queue within 15 minutes.
- Patients who DNA twice without notice receive a letter explaining re-booking procedure and consequences for care.
- Chronic disease patients (diabetes, hypertension, oncology) who DNA are contacted by nurse within 48 hours.

9. TELEMEDICINE & VIRTUAL OPD

CityHospital virtual OPD provides secure video-based consultations for eligible patients, reducing travel burden and maintaining care continuity for stable conditions.

9.1 Eligibility Criteria

- Follow-up consultations for clinically stable patients with an established diagnosis.
- Chronic disease prescription renewal where recent investigations are available in HIS.
- Post-discharge follow-up within 7 days for low-complexity cases.
- Patients who are geographically distant, elderly with mobility limitations, or at high infection risk.
- **NOT eligible:** New patients requiring physical examination, cases requiring procedures or urgent diagnostics, or paediatric patients under 5.

9.2 Virtual Consultation Workflow

Step 1. Patient books virtual appointment via hospital app or helpline; secure video link sent by SMS and email.

Step 2. OPD clerk confirms digital registration and sends pre-visit information sheet.

Step 3. Physician reviews EMR and all recent investigation results thoroughly before initiating the video call.

Step 4. Video consultation conducted on the hospital's HIPAA-compliant secure platform; consultation note completed in HIS during call.

Step 5. E-prescription issued via HIS; patient collects from OPD pharmacy or delivery to designated address arranged.

Step 6. If clinical assessment during the call indicates a need for in-person review, patient guided to urgent OPD booking.

Step 7. Telemedicine encounter documented as a standard OPD visit in HIS with encounter type marked as 'Virtual'.

WARNING: Telemedicine must not be used where the physician determines physical examination is clinically necessary. Patient safety always takes precedence over convenience.

10. DOCUMENTATION, EXCEPTIONS & BEST PRACTICES

10.1 Documentation Standards

- All OPD encounter notes must be completed in HIS before the physician ends the clinical session.
- SOAP note format mandatory for all clinical consultations.
- Referral letters completed same day and sent electronically with verified patient consent.
- Investigation results filed in HIS and linked to the originating OPD encounter number.
- Patient-declined investigations or medications: documented with clinical note and patient acknowledgement signature.
- Unsigned orders not permitted: all prescriptions, referrals, and investigation requests must be signed or electronically authenticated.

10.2 Complaint Handling

- Patient complaints received at OPD reception, patient relations desk, feedback kiosk, or online portal.
- All complaints acknowledged within 24 hours; resolution or escalation within 7 working days.
- OPD manager reviews aggregate complaint themes monthly; findings presented to department head quarterly.
- Staff against whom formal complaints are made are supported through HR and clinical governance processes.

10.3 Best Practices

- Apply the AIDET communication model in every patient interaction: Acknowledge, Introduce, Duration, Explanation, Thank You.
- Offer condition-specific patient education leaflets (top 20 OPD diagnoses) in the patient's preferred language.
- Use validated shared decision-making tools for chronic disease management: diabetes, hypertension, asthma, and lipid management.
- OPD digital satisfaction survey sent to each patient after every visit; results reviewed by department head monthly.
- Daily staff huddle at OPD opening and midday: workload review, special needs patients identified, and day's issues addressed.
- Actively monitor and address health literacy barriers; use plain language, visual aids, and teach-back confirmation.

10.4 Exceptions

- Paediatric OPD: parent or guardian must accompany; dedicated paediatric triage lane and waiting area provided.
- Unregistered walk-in patients requiring urgent care: triage nurse may initiate emergency referral directly, bypassing registration.
- HIS system downtime: manual paper log book activated; all entries back-entered into HIS within 4 hours of system restoration; downtime incident reported.
- Unaccompanied minors: safeguarding protocol initiated; social worker notified; care not withheld pending resolution.